

Operations Manual

Community-based Care Transitions Intervention

Harris County Area Agency on Aging

Revision History		
Date of Revision	Description of Change	Author
7/17/25	Patient Engagement : Engaging patients if non-POA family member is hesitant: 1) speak to patient directly; 2) describe collaboration b/n the orgs; 3) describe no additional costs; offer to connect to complex care team. Location: Engaging Patients	Marc
7/30/25	Medication Management : If HCAAA Health Coaches learn that a patient is being discharged to a SNF, and the patient is not aware of which SNF it will be, HCAAA Health Coaches will ask the St. Luke's Complex Care team during the daily huddle to look up which SNF.	Marc
8/5/2025	If HCAAA Health Coaches become concerned with patient's health trajectory, Health Coach will reach out to complex care team via email to make them aware of the health concerns and ask for a follow up from the complex care team for needed health intervention	Michelle
8/6/25	Provider Engagement : Physician letters will be sent to the St. Luke's Complex Care team in one batch email at the end of each week for all patients that have received a home visit within the previous seven business days.	Marc
8/26/2025	ACO Pulls Daily Report : If a patient has COVID, the St. Luke's team can still call the patient and obtain consent for the HCAAA Health Coaches to engage the patient and do a home visit once he is no longer contagious.	Marc
8/27/2025	4 Pillars : If the patient does not have their after visit summary during the home visit, the HCAAA Health Coach will ask the St. Luke's Complex Care team to review the patient chart and notify them of any necessary context (e.g. patient was not eating in hospital) or warning signs to look for (e.g. patient still not eating, feeling fatigued, etc.)	Marc
8/27/2025	Home visit preparation : When the HCAAA Health Coaches notify the St. Luke's Complex Care team of any home visits that are approaching, the St. Luke's Complex Care team will review the patient chart briefly to identify any warning signs to look for and communicate this to the HCAAA Health Coach.	Marc
8/27/2025	Red Flags : When in doubt, escalate any suspected signs of red flags back to the St. Luke's Complex Care team	Marc

Project Details	
Document owner	

Project Start Date	5/1/2025
Project End Date	4/30/2026
Project aims	1. Lower the rate of hospital readmissions
	2. Improve patient adherence to follow-up appointments with medical providers within 14 days of discharge
	3. Reduce patient's unmet social needs through improved social care coordination
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Harris County Area on Aging Project Stakeholders	Paula Johnson, Bureau Chief, Harris County Area Agency on Aging Dr. Janice Sparks, PhD, Director, CATCH

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In the Hospital

Step #	Step Description	Link Reference
1	Patient Identification Eligible patients will be shared with the Transition Coach (TC) through any of the following methods outlined in steps 1.1 - 1.3 once verbal consent to speak with the TC has been obtained by a member of the St. Luke's care team. Verbal Consent from the St. Luke's care team will be obtained through one of the following methods: 1) Accountable Care Organization (ACO) complex care team staff call recently admitted patients each day to introduce the program using this script and obtain verbal consent from the patient to have the TC speak with them. 2) Test virtual access connection with the patient 3) If the above two do not work, test in-person engagement a couple days per week	Verbal consent script
1.1	ACO Pulls Daily Report ACO complex care team pulls daily census report of admitted patients in Bamboo, allowing for a one day lag between admission and inclusion in the report in order to support caseload volume for the Transition Coach. The ACO team will only include patients in the daily report for whom the answer is "yes" to all three of the below questions. 1. Does the patient have original Medicare or Medicare Advantage insurance? a. If yes, continue to the next question to continue checking program eligibility. b. If no, the patient is not eligible for the program. 2. Is the patient admitted for an elective procedure? a. If yes, the patient is not eligible for the program. b. If no, continue to the next question to continue checking program eligibility. 3. Does the patient have a primary diagnosis of a serious mental illness or psychiatric condition? a. If yes, the patient is not eligible for the program. b. If no, the patient is eligible for the program and can be engaged in potential enrollment conversations.	TC Enrollment Checklist

Step #	Step Description	Link Reference
	ACO identifies “priority” patients that the TC should prioritize visiting. Patients deemed high priority are high utilizers. Note: If a patient has COVID, the St. Luke’s team can still call the patient and obtain consent for the HCAAA Health Coaches to engage the patient via phone and do a home visit once they are no longer contagious. There may be a group of patients who have been discharged outside of the 72 hours required for a home visit. Coaches will work with hospital staff to prioritize these patients to determine home visit timing. The ACO complex care team will also consider adding patients to this “priority” list who have recently been discharged from the hospital, but might benefit from the Harris County Area Agency on Aging (HCAAA) in-home supports to address health-related social needs.	
1.2	ACO Sends Daily Report ACO complex care team exports the daily report into an excel format and sends a secure email to the HCAAA group email inbox. The daily report will include the following information: patient first and last name, DOB, primary diagnosis, date of admission, and room location in hospital (manually added by ACO complex care team). It is assumed that all patients included in the report qualify for the program based on the three inclusion criteria identified in step 1.1. The TC opens the file each day and prioritizes ACO patients, starting with those that are flagged as priority by ACO.	
1.3	Daily Huddle Transition Coach (TC) and supervisor will participate in a daily huddle with a member of the St. Luke’s ACO complex care team each morning at X via phone/zoom. This huddle will review patients for both St. Luke’s Sugar Land Hospital and Baylor St. Luke’s Medical Center. The purpose of the daily huddle is to review any relevant information/cases from the previous day’s engagement and discuss any prioritized patients who have already provided verbal consent to engage for the upcoming day. During the huddle, the TC takes note of any patients who the complex care team members identify as requiring social care or care transitions support.	

Step #	Step Description	Link Reference
1.4	Care team referral Bamboo will state the unit location of the patient. Intensive care and psychiatric will be excluded. Care providers (VIC staff, Hospitalist, Chaplains, Social Workers, Admitting, Care Coordination) identify admitted patients requiring social care or care transitions support, and receive and document verbal consent via phone from the patient before referring to the TC.	
2	Health-related Social Needs (HRSN) Results Purpose: understand if a patient has social needs before engaging them in conversations about the program.	
2.1	TCs will view the HRSN screening results from the screening conducted by the hospital before engaging the patient. Before engaging a patient, to view the results from the HRSN screening conducted by the St. Luke's care team after admission to the hospital, the TC will do the following: 1. Log onto Greater Houston Connect at [INSERT URL]. 2. Enter patient first and last name, and DOB. 3. [INSERT ADDITIONAL STEPS]	
3	Patient Engagement Purpose: educate patients on opportunities for ongoing support post discharge and identify patients that want additional support.	
3.1	TC begins engaging patients, starting with patients that will be discharged soonest and working back from there. TC follows steps outlined in the patient engagement script. 1. If the patient declines enrollment, note the patient's name in Client Track and indicate that they declined. 2. If the patient agrees to enroll in the program, continue with the following steps which are outlined in the patient engagement script. a. Secure a signed consent form from the patient authorizing HCAAA to deliver their care transitions services. b. Discuss the patient's anticipated discharge date. c. Review with the patient what occurs during a home visit. d. Discuss potential dates for a home visit. e. Schedule the date of a home visit (or phone call or a virtual call if the patient does not want a home visit).	Patient engagement script

Step #	Step Description	Link Reference
	f. If the coach is unable to make contact with the patient, they will make two additional attempts to make the home visit. After three attempts, the patients is considered no longer interested in the program. g. If the patient will be discharged to a skilled nursing or rehab facility, ask the patient for a potential discharge date from the facility, and make contact with the patient at least two times a week until discharged from the facility. Enter patient DOB and full name into Client Track. h. Family members: if a patient has a family member that is not a Power of Attorney, but is hesitant about their loved one enrolling in the program, the Health Coach should reinforce the following messages: i. Speak directly with the patient. ii. Emphasize that St. Luke's and AAA are collaborating. iii. Emphasis that these are AAA services and Medicare is not billed, and any costs (or no costs) incurred by the patient. iv. If the family has additional questions, offer to answer any questions they may have. Describe any AAA services that may be helpful, such as caregiver services. i. After the patient has been discharged from the facility, follow the home preparation steps starting with item 2.	
4	Home Visit Preparation Purpose: The Transitional Coach identifies patients to complete post-discharge home visits with and begins assessments and integrating wrap-around care.	
4.1	The TC completes the following steps to prepare for that afternoon's home visits: 1. Enter patient DOB and full name into Client Track. 2. Reviews the health-related social needs screening results available in Houston Health Connect. Based on results of the hospital's HRSN screening, TC marks in Client Track the appropriate HRSN response options in client intake steps. These will be responses from the AHC screening. 3. Notifies the St. Luke's Complex Care team of the home visits that day or in the next couple of days so that St. Luke's Complex Care team can review the patient chart briefly to identify any warning signs to look for and communicate this to the HCAAA Health Coach.	

Step #	Step Description	Link Reference

30 Day Care Transitions

Step #	Step Description	Link Reference
5	Patient Discharges from Hospital and TC offers follow-up care: • Within 72 hours post-discharge, the TC visits patients at home that are enrolled in the program to perform screenings and supports the patient in completing a Personal Health Record for use in their plan of care. • The TC team prioritizes visiting with patients who have been discharged the longest from the hospital. • TC conducts these visits in the morning or afternoon, whichever one works best for the patient.	
6	Home Visit Screenings Purpose: complete three evidence-based screenings with patients during the initial post-discharge visit. The screenings are a health-related social needs (HRSNs) screening, falls risk screening, and Patient Health Questionnaire PHQ-2 and/or PHQ-9.	
6.1	Patient Health Questionnaire Purpose: screening tool used to identify depression. 1. In the assessments tab of Client Track, the TC asks the two questions outlined in the standard Patient Health Questionnaire-2. Does the client score 3 or greater on the PHQ-2? a. If yes, the TC documents the score in Client Track and administers the full PHQ-9. The TC sends a secure email with the patient score for patients that score 10 or over - moderate (10-14), moderately severe (15-19), or severe (20-27) high- to all of the St. Luke's Complex Care team email addresses listed in Appendix B. TCs will continue to work with clients with moderate to high scores, but will be in a supportive role to the ACO's complex care team since the medical needs will likely take highest priority rather than social needs. b. If no, the TC documents the score in Client Track and does not administer the full PHQ-9. No further action is required. 2. For patients that communicate suicidal ideation, the TC will call 911, stays with the client until EMS support arrives, emails all of the St. Luke's Complex Care team email addresses listed in Appendix B, and makes a note in Client Track.	PHQ-2 PHQ-9

Step #	Step Description	Link Reference
6.2	Falls Risk Assessment Purpose: To assess fall risk, the TC will ask the following targeted questions to the client. 1. The TC will review the bathroom condition with the client to make sure it's safe. 2. The TC will look for anything that could lead to a slip, trip, or fall. 3. The TC will ask the client the following questions and document responses in Client Track: a. Do you have any difficulty getting in and out of the bathtub or shower? b. Have you ever slipped or felt unsteady while using the bathroom? c. Do you have any trouble reaching for items like soap, shampoo, or towels? d. Do you feel safe and comfortable using the toilet? 4. The TC reviews the patient's medication list, documents medications in Client Track, and informs the patient if any medication indicates mobility or potential dizziness side effects. If the patient has a question about their medication(s), or duplicate medications are being taken, the TC will call a St. Luke's complex care team staff member with the patient to ask the question. After the call, the TC will follow up with an email to the complex care team member to summarize the action plan (email addresses listed in Appendix B). 5. TC writes any notes or recommendations and then marks Falls Risk Assessment complete in Client Track.	
6.3	Health Related Social Needs Purpose: Confirm with the client the results of the HRSN screening conducted by the hospital and ensure the client does not need assistance with additional HRSN. In preparation for the home visit, the TC should have reviewed the client's HRSN screening results in Houston Health Connect and checked off those results in the client's Client Track intake form (see step 4.1). During the post discharge visit, the TC completes the following steps to assess the client's HRSNs. 1. Open Client Track 2. Review the intake form HRSN results that should have been already marked off based on the hospital screening	HRSN Screening Tool

Step #	Step Description	Link Reference
	3. Complete the following HRSN questions in Client Track: a. Did you confirm with the client the results of the HRSN screening they received in the hospital? b. Would the client like assistance addressing any HRSNs for which they screened positive? c. Are there any additional social needs the client would like assistance addressing? 4. If the client has any HRSNs for which they'd like assistance, the TC will develop a person-centered plan with the client to address and document it in Client Track.	
7	4 Pillars Purpose: To empower clients to manage their health and communicate effectively with their healthcare providers. The coach will give the client a copy of their personal health record (PHR) to record follow-up activities.	
7.1	Personal Health Record Purpose: The Personal Health Record (PHR) is completed with information that the patient can use to manage and track their health. The PHR enables the patient to centralize and identify questions to ask their provider before or during the next appointment. The TC encourages the patient to self-complete the PHR and answer the following questions, but assists the patient in filling it out when necessary. 1. The client and/or TC fill out the following in the PHR workbook: a. Patient, caregiver, and provider information b. What the patient plans to do to manage their health and medications. c. Questions the client has for their PCP. d. List any personal notes, such as personal goals, or hospitalization notes e. Medical history and red flags. f. Medication and supplement record. i. Names of medication (dose, reason, and if new or not). ii. Allergies iii. Any notes or questions for PCP. 2. TC marks tasks completed in Client Track. 3. The TC encourages the client to bring their PHR to appointments and update it frequently as they receive new information. 4. The TC coordinates with community-based resources to help support patient and reduce roadblocks in patient follow-up care.	Personal Health Record

Step #	Step Description	Link Reference
	Note: If the patient does not have their after visit summary during the home visit, the HCAAA Health Coach will ask the St. Luke's Complex Care team to review the patient chart and notify them of any necessary context (e.g. patient was not eating in hospital) or warning signs to look for (e.g. patient still not eating, feeling fatigued, etc.)	
7.2	Medication Management Purpose: Help the patient identify their medications and formulate any questions they want to ask their healthcare provider at the follow-up appointment. 1. TC reviews the following items with the client: a. Compare pre-hospital medications with medications on hospital or skilled nursing facility (SNF) discharge list. b. Identify medications that were prescribed but not obtained c. Identify medication discrepancies d. Call the complex care team to resolve discrepancies e. Encourage or observe use of patient's medication management "system" (e.g., Mediset ® or paper chart or alarm clock) f. Identify medications needing refills and/or barriers to refill g. Other 2. If patients indicate challenges paying for medications, TCs should initiate a referral to the State Health Insurance Prescription counselor at AAA. 3. Once the above have been reviewed, the TC marks the activities as completed in Client Track and makes note in their PHR. <i>Note: For patients that are discharged to a SNF, TC will contact the SNF to inform them of the care transitions program support that the patient will receive once they are discharged from the SNF. This may require the TC to engage with the SNF's social worker on a weekly basis. If the social worker is unresponsive, the TC can ask the HCAAA ombudsman to support a connection to the social worker. The patient's after visit summary provided by the hospital may also include a name and contact person from the SNF.</i>	
7.3	Medical Care Follow Up Purpose: Support the patient in successfully adhering to follow-up appointments with their healthcare provider(s). 1. TC reviews the following items with the client.	

Step #	Step Description	Link Reference
	a. Encourages the client to set up follow-up appointment b. Role-play appointment scheduling and encounter, or support client in calling their provider c. Identify problems that require immediate PCP or specialist visit d. Assist the client in accessing transportation support (if necessary) e. Develop questions with client for PCP or specialist f. Teach skill of writing questions to ask at PCP or specialist follow up g. Clarify whether client will need to obtain follow up tests and/or results; ask patient if they have access to their online medical record and encourage them to follow up with their PCP. h. Provide teaching for how to obtain follow-up tests and results i. Other	
7.4	Red Flags Purpose: Educate the client about “red flags” or warning signs/symptoms and how to respond. 1. TC reviews the following items with the client a. Review discharge instructions b. Discuss & teach self-management of condition(s); TC will review health education materials that explain health conditions and warning signs of possible problems related to condition. Education materials can be access on the City of Houston G Drive. c. Discuss target symptoms / side effects to monitor and what to do should they arise d. Discuss when PCP should be called and specific plan of how to contact clinicians by providing contact numbers, including evenings and weekends. e. Instruct on what constitutes an emergency and what to do in cases of emergency and nonemergency situations. f. Refer the patient back to the complex care team and/or primary care provider for questions g. When in doubt, escalate any suspected signs of red flags back to the St. Luke’s Complex Care team 2. Once the above have been reviewed, the TC marks the activities as completed in Client Track.	Educational Materials (available in AAA drive)

Step #	Step Description	Link Reference
8	Provider Engagement Purpose: Promote continuity of information amongst the patient, healthcare provider, ACO complex care team, and HCAAAA. - At the end of each week, the TCs will send the physician update letter in one batch for all patients who have received a home visit within the previous 7 days. The following steps should be completed to send the letters. - Access the template physician letter hyperlinked in the “link reference” column. - Create a copy that will be specific to the client. - Customize the information in the letter based on the activities conducted and performed on behalf of the client. - Send the letter to all of the St. Luke’s Complex Care team email addresses listed in Appendix B. - The ACO team will include the letter in the patient’s medical chart for the provider to view. - The TC marks in Client Track that the physician letter has been completed.	Sample Physician Letter
9	30 Day Follow-up Purpose: Understand client’s success in accessing follow-up care and any ongoing support needed to address social needs. 30 days after the client’s discharge from the hospital, the TC calls the client. Make at least three phone call attempts to reach the client. - If the client answers, the TC: - Marks the date of their phone call. - Review the personal health record to determine progress towards goals on the 4 pillars and document in client answers in Client Track. - Asks the client the full set of HRSN screening questions that are being used by St. Luke’s Health (7 questions) and marks the appropriate responses. - Did the client screen positive for any of the HRSN screening questions? - If yes, the TC asks the client if they’d like support for an additional 30 days in addressing the HRSN(s) for which they screened positive? If the client would like 30 more days of support, the TC updates the person-centered plan to reflect 30 more days of targeted support.	

Step #	Step Description	Link Reference
	b. If no, the TC marks the appropriate HRSN screening responses in Client Track and moves to administering the program completion survey (step 10.3). 4. If the client does not answer, the TC marks the date of their phone call attempts and marks the appropriate box in Client Track to indicate the client's completion of the 30-day care transitions program.	

Ongoing Wrap-Around Services (Days 31-90)

Step #	Step Description	Link Reference
10	60 Day Assessment Purpose: To address the patient's ongoing health-related social needs. 60 days after discharge, the TC calls the client and completes steps 10.1 - 10.3. The TC will make at least two attempts to reach the client via phone.	
10.1	Personal Health Record Purpose: assess how the client is progressing towards any documented goals. 1. The TC opens up Client Track to review any previous action items and goals with the client. 2. The TC makes any relevant notes in Client Track to the status of the goals and/or previously identified needs	
10.2	Health Related Social Needs Purpose: Identify ongoing HRSNs affecting the client. The TC completes the following steps: 1. The TC administers the full HRSN screening available in Client Track (7 questions) and marks the appropriate responses. 2. Did the client screen positive for any of the HRSN screening questions? c. If yes, the TC asks the client if they'd like support for an additional 30 days in addressing the HRSN(s) for which they screened positive? If the client would like 30	

Step #	Step Description	Link Reference
	more days of support, the TC updates the person-centered plan to reflect 30 more days of targeted support. d. If no, the TC marks the appropriate HRSN screening responses in Client Track and moves to administering the program completion survey (step 10.3).	
10.3	Program Completion Survey Purpose: assess client satisfaction with the care transition and case management services provided. The AAA will partner with Blooming Health to conduct client satisfaction surveys. The TC completes the following steps: 1. TC will log into Blooming Health to access and complete the survey with the patient. 2. Blooming Health will share the results with CT staff. 3. If a patient expresses concerns or experienced difficulties with a coach or the program, the appropriate people will be notified and discuss the matter to develop a resolution.	FAQ
11	90 Day Assessment Purpose: To address the patient's ongoing health-related social needs. 90 days after discharge, the TC calls the client and completes steps 11.1 - 11.4. The TC will make at least two attempts to reach the client via phone.	
11.1	Personal Health Record Purpose: assess how the client is progressing towards any documented goals. 1. The TC opens up Client Track to review any previous action items and goals with the client. 2. The TC makes any relevant notes in Client Track to the status of the goals and/or previously identified needs.	
11.2	Health Related Social Needs Purpose: Identify ongoing HRSNs affecting the client. The TC completes the following steps: 1. The TC administers the full HRSN screening available in Client Track (7 questions) and marks the appropriate responses. 2. Did the client screen positive for any of the HRSN screening questions? 3. If yes:	

Step #	Step Description	Link Reference
	a. The TC marks the appropriate HRSN screening responses in Client Track. b. The TC updates the person-centered plan to reflect the unresolved status of previously identified HRSNs. c. The TC tells the client that they will notify the healthcare team of the ongoing needs. d. The TC then sends a secure email to the ACO complex care team sharing the client's first name, last name, DOB, and ongoing HRSNs that are unresolved, and the date the client reached full program completion (90 days post discharge). 4. If no: a. The TC marks the appropriate HRSN screening responses in Client Track and moves to administering the program completion survey (step 11.3).	
11.3	Program Completion Survey Purpose: assess client satisfaction with the care transition and case management services provided. The AAA will partner with Blooming Health to conduct client satisfaction surveys. The completes the following steps: 1. Step 1TC will log into Blooming Health to access and complete the survey with the patient. 2. Blooming Health will share the results with CT staff. 3. If a patient expresses concerns or experienced difficulties with a coach or the program, the appropriate people will be notified and discuss the matter to develop a resolution.	FAQ
	Completion of CBCT Program The weekly/monthly spreadsheet sent from the AAA to the ACO complex care team will include a program completion date for clients that have finished the program.	

Appendix A: Hospital-based Coach's Daily Schedule (Sample)

Time	Activity	Key Responsibilities	Location
8:00am	ACO Report/Chart Review	TC opens daily ACO report to identify patients flagged as priority by ACO and to understand primary diagnoses. TC accesses patient chart in Houston Health Connect to understand health-related social needs screening results	Hospital 1*
8:30am	Daily Huddle	TC and supervisor participate in a daily huddle with a member of the St. Luke's ACO complex care team to review any relevant information/cases from the previous day's engagement and discuss any prioritized patients who have already provided verbal consent for the upcoming day (Step 1.3).	Hospital 1
9:00am	Patient engagement	TC visits patients to discuss enrollment in care transitions program. (Step 3)	Hospital 1
12:00pm	Lunch		
1:00pm	Travel to Hospital 2		Car
2:00pm	Patient engagement	TC visits patients to discuss enrollment in care transitions program. (Step 3)	Hospital 2
4:00pm	Documentation	TC documents any pending notes/information in Client Track for clients seen that day, if not completed already; communicates with Home-visit coach regarding patients	Hospital 2
5:00pm	End of day		

*Hospital-based coach will alternate where they begin the morning between both hospitals to complete ACO Report/Chart Review.

Home Visit Coach's Daily Schedule (Sample)

Time	Activity	Key Responsibilities	Location
8:00am	Coach prepares/completes documentation and data entry	Coach completes any documentation in Client Track from the previous day, makes phone calls, coordinates meetings and services.	Home office
9:00am	Home visits	TC spends the morning engaging clients in 30 day care transition protocol or Ongoing Wrap-around Services, depending on client. (Steps 5 - Steps 10.3)	Client homes
12:00pm	Lunch		
1:00pm	Home visits		Client homes
3:00pm	Travel	TC travels to home office to prepare for afternoon documentation and service coordination	Car
3:30pm	Document home visit activity	TC documents activities from home visits in Client Track and coordinates meetings, services, etc.	
5:00pm	End of day		

Appendix B: Project Support Team

Name	Title	Email Address	Phone Number	Area of Support
Escalation Pathway				
Dr. Janice Sparks, PhD	Director, CATCH	pdcatch@ibnsinafoundation.org	832-395-8927	AAA Program Oversight
Paula Johnson	Bureau Chief, Harris County Area Agency on Aging	paula.johnson@houstontx.gov	832-393-4619	AAA Program Oversight
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Michelle Caro	CHW	michlle.caro@houstontx.gov	832-707-7331	Transition Coach Lead
Elizabeth Stuart	CHW	elizabeth.stuart@houstontx.gov	346-845-5551	Transition Coach
Esmeralda Calderon	CHW	esmeralda.calderson@houstonx.gov	832-431-2659	Transition Coach
Porche Turner	CHW	porche.turner@commonspirit.org	346-329-0809	
Tracy Maddox, JD, MSN, RN	Texas CIN Division Director, Quality-CIN Administration	Tracy Maddox TX-Houston	832.355.2670	Baylor St. Luke's clinical services.
St. Luke's Complex Care Members/Email Addresses	Deborah Dornes - deborah.dornes@commonspirit.org Marlo McManus - marlo.mcmanus@commonspirit.org Brandi Dorsey - brandi.dorsey@commonspirit.org Porche Turner - porche.turner@commonspirit.org			Escalating or sharing patient information back with St. Luke's care management team.

Additional Project Stakeholders				
Megan Harkey, MHA	Texas Markets Vice President, Clinically Integrated Network	Megan Harkey TX-Houston		
Marc Rosen, MPH	System Director, Community Impact and Partnerships	Marc Rosen -IL	224-558-5182	Support reaching St. Luke's contacts
Corrine Barfield	Executive Assistant, Sugar Land Hospital CEO	corinne.barfield@commonspirit.org	281-637-7600	Finding work space at Sugar Land Hospital
Devin Bradberry	Senior Staff Analyst for the Houston Health Department	Devin.Bradberry@houstontx.gov	832-393-4901	Client Track
Prudhvi Ravipati	Houston Health Department	prudhvi.ravipati@houstontx.gov	832-393-4980	Client Track

Appendix C: Frequently Asked Questions: Transition Coach Sample Responses

Q: Can the TC continue providing support services to the client after 90 days post-discharge?

A: Program funding only allows for clients to receive up to 90 days of services. If the client has new or emergent medical needs at 90 days post-discharge, the TC can securely communicate those medical needs or HRSNs back with the ACO team, and also explore a connection to the Aging and Disability Resource Center to see if they can continue assisting the client.